Executive summary: Translate MNC activity into testable project concepts

Proposed opening page | Three observed shifts; each leads to a specific internal decision.

01 OBSERVED SHIFT

Build combinations around a defined treatment backbone

AZ: Tagrisso-based combinations

J&J: TECVAYLI + TALVEY

Roche: divarasib + pembrolizumab

QUESTION FOR OUR PORTFOLIO

Which patient segment remains poorly served by the next combination standard?

Screen one biomarker-defined combination concept; specify the relevant comparator.

02 OBSERVED SHIFT

Use delivery to solve a measurable clinical limitation

AZ: inhaled anti-TSLP Fab

J&J: intravesical gemcitabine

Roche: Aβ / TfR1 Brainshuttle

QUESTION FOR OUR PORTFOLIO

Can exposure, safety or treatment burden be improved for a target we can access?

Define the exposure or usability benefit required before selecting a platform.

03 OBSERVED SHIFT

Separate new technology access from a viable product

ADC, TCE and degradation approaches appear across the three profiles; stages and applications differ.

QUESTION FOR OUR PORTFOLIO

Does the modality address a target-specific limitation that simpler options cannot solve?

Compare internal build, asset licensing and platform access for the same product concept.

Source: user-provided vMaster PDF, pp. 5–10, 13–18, 21, 23–27. Pilot synthesis; not a 20-company finding.

Cross-company signals: Identify convergence, then test its relevance

Presence matrix | A filled cell means a specific example is documented; a blank is not proof of absence.

Theme	Johnson & Johnson	Roche	AstraZeneca
Combination / earlier-line development	TECVAYLI + TALVEY BCMA/CD3 + GPRC5D/CD3 TCEs	Divarasib + pembrolizumab KRAS G12C inhibitor + anti-PD-1	Tagrisso + Datroway EGFR TKI + TROP2 ADC
Delivery / route innovation	INLEXZO Intravesical gemcitabine	Trontinemab Aβ / TfR1 Brainshuttle	AZD8630 Inhaled anti-TSLP Fab
ADC portfolio investment	JNJ-0683 HER2 ADC	RG6975; RG6810 CDH17 ADC; DLL3 ADC	AZD8205; AZD5335 B7-H4 ADC; FRα ADC
Cell therapy / B-cell targeting in immune disease	Sail collaboration In-vivo CAR-T; target not disclosed	Gazyva; bexobrutideg Anti-CD20; BTK degrader	CAR-T / TCE autoimmune trials Programs discussed by management
Multi-mechanism metabolic treatment	— Not captured in this pilot	Enicepatide + petrelintide GLP-1/GIP agonist + amylin analogue	GLP-1 / SGLT2 development GLP-1 / PCSK9 remains exploratory

Interpretation: convergence identifies a question to investigate. It does not establish differentiation, market demand or technical feasibility.

Source: vMaster PDF, J&J pp. 5, 6, 8–10; Roche pp. 13, 14, 17, 18; AZ pp. 21, 23, 25–27.

Opportunity shortlist: Define the clinical problem before choosing a target

Illustrative hypotheses for internal screening | No project is ranked until internal fit and external differentiation are established.

A | Oncology combinations

GI cancer: biomarker-defined ADC–IO combination

EXTERNAL SIGNAL

AZ is developing CLDN18.2 / TROP2 ADC combinations with PD-1/VEGF; Roche lists a CDH17 ADC.

DIFFERENTIATION TO PROVE

An unserved subgroup, resistance state or tolerability window versus the expected combination standard.

INTERNAL FIT TO CHECK

Relevant internal asset or credible BD access; translational assays; combination safety.

B | Targeted / local delivery

Improve exposure for one asset with a known clinical limitation

EXTERNAL SIGNAL

Inhaled anti-TSLP, intravesical gemcitabine and Brainshuttle show different ways to change exposure.

DIFFERENTIATION TO PROVE

A measurable tissue-exposure, therapeutic-index or administration advantage.

INTERNAL FIT TO CHECK

An internal molecule with a delivery bottleneck; tissue PK; formulation and CMC access.

C | Immune disease expansion

B-cell targeting with a safety / access advantage

EXTERNAL SIGNAL

J&J / AZ discuss CAR-T in immune disease; Roche is extending B-cell-directed development.

DIFFERENTIATION TO PROVE

Durable disease control with acceptable infection risk, treatment burden and cost.

INTERNAL FIT TO CHECK

Immune-disease strategy; clinical access; cell-therapy or alternative modality capability.

Source: vMaster PDF pp. 10, 14, 18, 27. Internal hypotheses below are illustrative, not management guidance.

Project concept example: Test an ADC–IO hypothesis in a defined GI population

Illustrative discussion card | Decision requested: approve a focused feasibility assessment, not a full development program.

PRODUCT HYPOTHESIS

A tumor-selective ADC + IO regimen for one GI cancer subtype with a defined unmet need.

Choose antigen, population and line together. Use CLDN18.2 / TROP2 combinations as competitive benchmarks, not default project choices.

MINIMUM EVIDENCE PACKAGE

1 Patient and competitor map	Select one cancer / line; assess antigen heterogeneity, resistance biology and expected standard of care.
2 Mechanism and exposure	Compare ADC, IO and the combination in immune-relevant models; link response to biomarker and exposure.
3 Safety and feasibility	Test normal-tissue / immune-cell effects; check combination exposure, CMC, IP and access to both agents.

DECISION GATES

ADVANCE

Distinct patient rationale + credible combination benefit at feasible exposure + an accessible asset route.

REVISE

Biology is attractive, but indication, partner or delivery needs to change.

STOP

No advantage over the relevant comparator; no usable safety window; no practical asset / IP access.

Set quantitative thresholds before experiments.

Source: external context from vMaster PDF pp. 14, 27. Concept, experiments and gates are proposed for discussion.

Data architecture: Reuse each company page as structured evidence

One evidence register → comparable company profiles → cross-company themes → project concept cards.

Existing page	Fields to extract / add	Cross-company output	Input to project decisions
Commercial portfolio	Product / TA sales, growth; add US LOE and replacement-program links	Growth engines and replacement needs (no gap inferred from sales alone)	Indication value; timing of opportunity
Late-stage portfolio	Asset + indication + line + stage; NME / LCM; combo; territory	Next standards of care; crowded and under-served patient segments	Future comparator; required TPP
Early-stage pipeline	Target × modality × indication; biomarker; delivery features	Emerging mechanisms and technical approaches	Mechanism options; validation needs
BD / M&A	Announce date; deal type; acquired rights; asset stage; upfront vs milestones	Where companies source assets / capabilities externally	Build / license / collaborate route
R&D priorities / calls	Specific action; named program; timing; source location; committed vs exploratory	Forward agenda and catalysts to monitor	What to test now; when to reassess

Add an internal overlay: assets / assays / models / clinical access / CMC / IP / budget. This is the missing bridge from trend to project.

Source: proposed analytical framework, aligned to the existing company-page structure. No new numerical aggregation shown.

Portfolio review: Move from a theme to a named owner and a decision

Proposed review process | Evidence quality and internal fit determine the next step; popularity is not a score.

01 Screen the theme

Is there a defined patient need and an evidence-based external signal?

Output: opportunity brief

Proposed participants: Strategy + Clinical

02 Test internal fit

Can we access a relevant asset and generate decisive evidence?

Output: feasibility plan / BD route

Proposed participants: Preclinical + BD + CMC

03 Decide the project

Is the product differentiated, feasible and worth the required investment?

Output: advance / revise / stop

Proposed participants: Portfolio committee + PMO

FIRST REVIEW CYCLE | Illustrative six-week assessment

Weeks 1–2

Resolve source / scope issues; define patient need and future comparators.

Weeks 3–4

Map internal assets and capabilities; design experiments; check partner access.

Weeks 5–6

Submit 1–2 concept cards with evidence, gaps, resources and a decision request.

Source: proposed internal workflow. Owners and timing are illustrative, to be agreed with the team.

Evidence discipline: Make every recommendation traceable to a source

Appendix | Suggested tags: observed action / management plan / analyst hypothesis / internal decision.

PILOT EVIDENCE MAP	
Combination / earlier lines	J&J p.10; Roche p.18; AZ p.27
Delivery / route innovation	J&J p.10; Roche p.18; AZ p.27
ADC investment	J&J pp.5–6; Roche p.14; AZ pp.21, 23, 27
Immune-disease expansion	J&J p.10; Roche p.18; AZ p.27
Metabolic combinations	Roche p.18; AZ p.27

Carry forward original IR / transcript / registry URLs, source dates and exact passages into the final evidence register. The supplied PDF is the source used for this example.

BEFORE SCALING TO 20

Align snapshot dates, TA mapping and NME / LCM definitions.

Deduplicate molecules, indications and transactions separately.

Do not count Genentech / Chugai group provenance as a new recent M&A.

AZ p.22 still repeats Roche early-stage content; exclude it from AZ comparisons.

Source: user-uploaded “Top 20 MNC Portfolio and Pipeline Strategy Intelligence_vMaster.pdf”, 28 pages.